

Where would extra listening change a decision?

Discussion proposal for NHS workforce and staff-experience analysts

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Not co-designed with NHS staff; no partnership, prospective study or deployment has taken place.

Decision. Before a scheduled people-review meeting, allocate a limited budget for additional information gathering, then identify units needing further wellbeing review. Preserve universal access to the existing annual and quarterly feedback channels. The tool does not allocate wellbeing services, diagnose staff, or infer an intervention's benefit.

Proposed users and output. A workforce/OD analyst prepares an uncertainty-aware shortlist with a staff-experience lead. The responsible people leader, staff representatives and survey provider review the proposed follow-up. Output: candidate units, reason for uncertainty, suggested information channel, expected information gain under stated assumptions, estimated cost, and a record of human changes. These roles and workflow are proposed, not confirmed by a partner.

What the current research can supply. Historical NHS organisation scores support a scenario demonstration. They are not department data or observed resurvey outcomes. Separate BRFSS individual-record replay tests acquisition on recorded US health responses; it is not NHS burnout validation. A second survey is not automatically unbiased: shared nonresponse, measurement differences and time trends must be considered.

Prospective evaluation proposed before any rollout

- 1. Agree the task and costs.** With a real partner, fix the review unit, meeting date, available follow-up budget and decisions it can change. Record invitation, completion, analysis and staff-time costs separately. Confirm questionnaire permissions, voluntary participation, confidentiality and reporting rules. No budget or effect size is assumed to be an NHS estimate.
- 2. Run in shadow mode.** Freeze candidate policies and their inputs before the next measurement wave. Compare matched-budget uniform follow-up, a simple uncertainty rule, and the proposed cost-aware policy; keep decision-makers' usual process. Log recommendations without directing services or treating nonresponse as a negative answer.
- 3. Obtain a separate evaluation sample.** Use a partner-approved probability sampling and follow-up design, independent of policy recommendations where feasible. Record overlap, response probabilities, weights, item correspondence and timing. It is a reference estimate with uncertainty, not an unquestionable gold standard. Small cells remain protected, including against identification from repeated releases.
- 4. Assess information and usability.** Predefine the primary measure as improvement in the agreed review decision per unit of actual resource used. Report uncertainty, errors near the selection boundary, subgroup coverage, cost overruns, analyst time, staff burden and human overrides. Preserve reversals and inconclusive results. Choose sample size and thresholds with the partner using precision and feasibility calculations before seeing evaluation outcomes.

Decision after evaluation. Continue only if the information is sufficiently reliable, the workload is acceptable and staff representatives and decision owners find the output useful. Keep existing escalation routes for serious concerns. A future wellbeing-outcome trial would require a separate intervention and causal evaluation design.

Official basis: [NHS Staff Survey FAQ](#); [Listening well](#); [NQPS guidance](#). Annual burnout and quarterly engagement are distinct measures. This proposal has not been sent to an NHS organisation.